

**Question:**

A baby is born after the 32nd gestational week by cesarean delivery. The mother suffered from gestational diabetes; however, she had no other pregnancy-related diseases and was otherwise healthy. The baby has a blood pressure of 100/58 mm Hg, heart rate of 104/min, and oxygen saturation of 88%. The child has tachypnea, subcostal and intercostal retractions, nasal flaring, and cyanosis. The cyanosis is responding well to initial administration of oxygen. The nasogastric tube was positioned without problems. Which of the following is the most likely diagnosis?

Answer Choices:

- A. Neonatal respiratory distress syndrome (NRDS)
- B. Tracheoesophageal fistula
- C. Pneumonia
- D. Congenital heart anomaly with right-to-left shunt

**Reasoning + Answer:**

<think> Reasoning process here : The baby was born at 32 weeks gestation , which is considered pre term . However , it is not so premature that it would immediately put the baby at risk for respiratory distress syndrome ( A ). The presence of tachypnea , sub cost al and inter cost al retractions , nasal fl aring , and cyan osis suggests respiratory distress , but the baby responds to oxygen . A nas oga stric tube placement was also successful , which helps to rule out a Trache oes ophage al fistula ( B ). The high oxygen saturation levels do not support pneumonia ( C ) as a primary diagnosis . Therefore , the most likely diagnosis is a congen ital heart anomaly with a right -to -left sh unt that has led to respiratory distress ( D ), such as hyp oplasia of the lung tissue . </think>< answer >D . Cong en ital heart anomaly with right -to -left sh unt </answer>

